



Overdose/ Toxic Ingestion

History

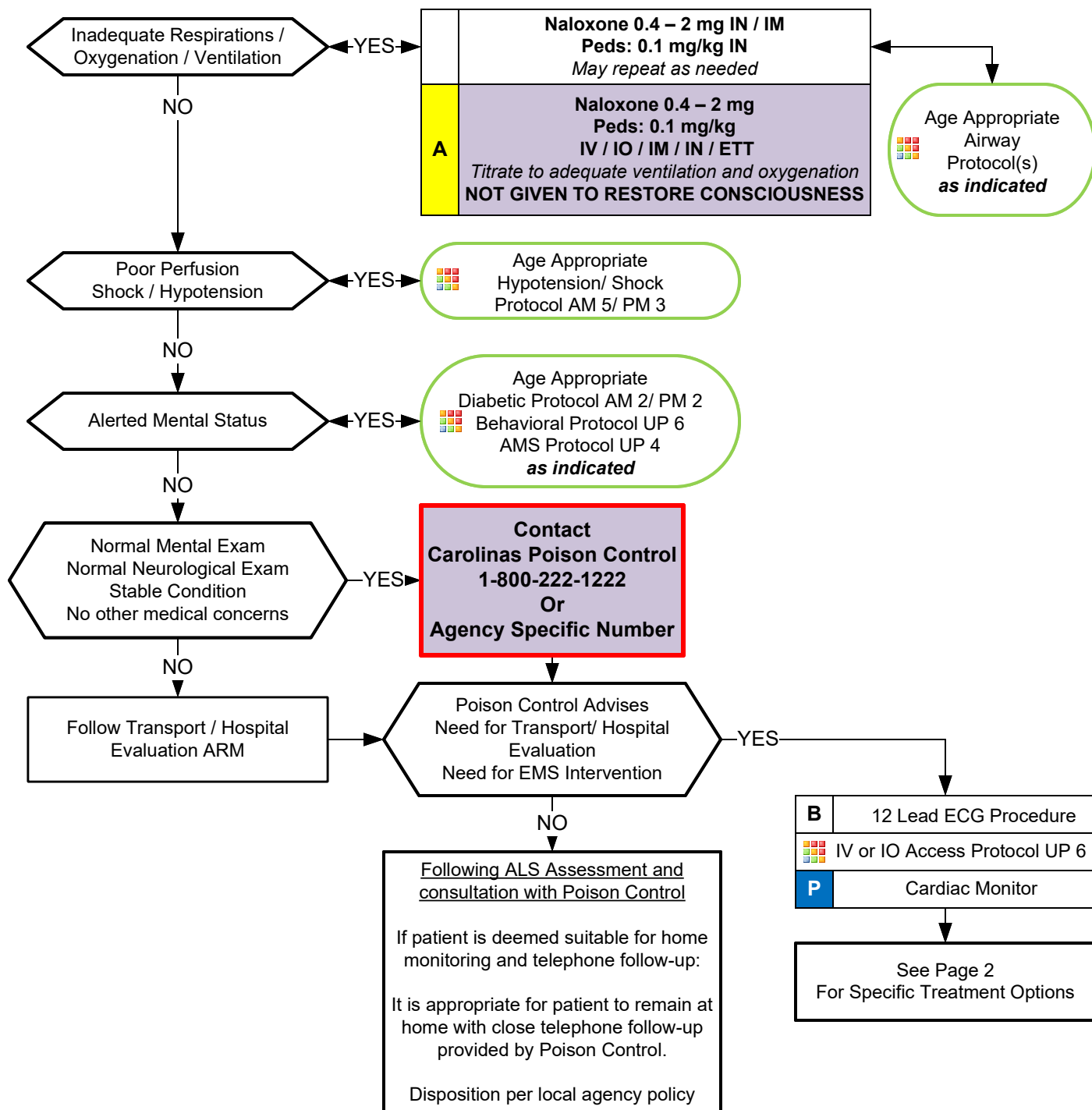
- Ingestion or suspected ingestion of a potentially toxic substance
- Substance ingested, route, quantity
- Time of ingestion
- Reason (suicidal, accidental, criminal)
- Available medications in home
- Past medical history, medications

Signs and Symptoms

- Mental status changes
- Hypotension / hypertension
- Decreased respiratory rate
- Tachycardia, dysrhythmias
- Seizures
- S.L.U.D.G.E.
- D.U.M.B.B.E.L.S

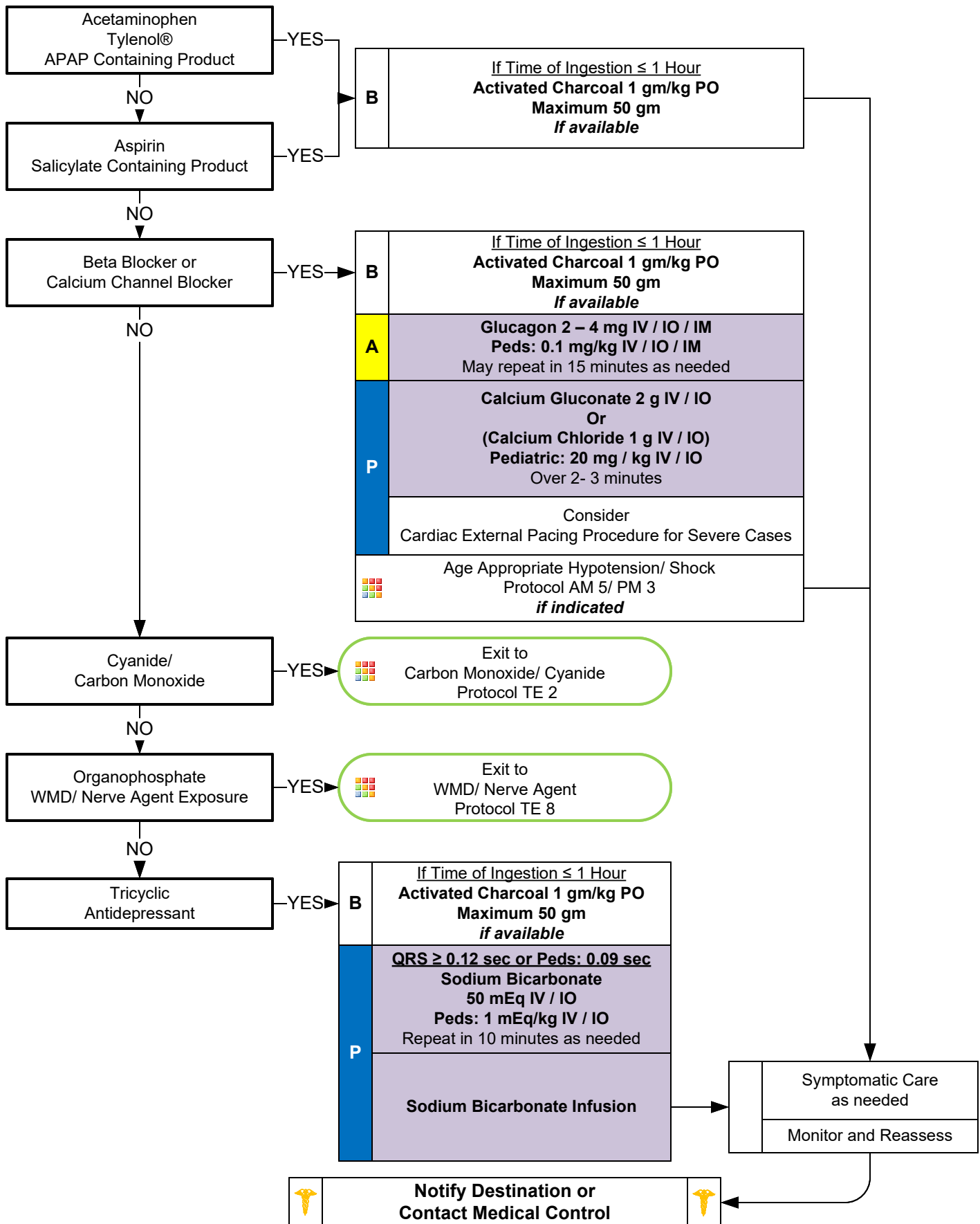
Differential

- Tricyclic antidepressants (TCAs)
- Acetaminophen (Tylenol)
- Aspirin
- Depressants
- Stimulants
- Anticholinergic
- Cardiac medications
- Solvents, Alcohols, Cleaning agents
- Insecticides (organophosphates)





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Pearls

- **Recommended Exam: Mental Status, Skin, HEENT, Heart, Lungs, Abdomen, Extremities, Neuro**
- **Opioids and opiates may require higher doses of Naloxone to improve respiration, in certain circumstances up to 10 mg.**
- **Time of Ingestion:**
 1. **Most important aspect is the TIME OF INGESTION, the substance(s), amount ingested, and any co-ingestants.**
 2. **Every effort should be made to elicit this information before leaving the scene.**
- **Charcoal Administration:**

The American Academy of Clinical Toxicology DOES NOT recommend the routine use of charcoal in poisonings.

 1. Consider Charcoal within the FIRST HOUR after ingestion. If a potentially life threatening substance is ingested or extended release agent(s) are involved and $\geq$ one hour from ingestion, Contact Medical Control or NC Poison Control Center for direction.
 2. If NG would be necessary to administer Charcoal, then DO NOT administer unless known to be adsorbed, airway secured by intubation, and ingestion is less than ONE HOUR confirmed and potentially lethal.
 3. Charcoal in general, should only be given to a patient who is alert and awake such that they can self-administer the medication.
- **Do not rely on patient history of ingestion, especially in suicide attempts. Make sure patient is still not carrying hiding other medications or has any weapons.**
- **Pediatric:**

Age specific blood pressure 0 – 28 days > 60 mmHg, 1 month - 1 year > 70 mmHg, 1 - 10 years > 70 + (2 x age)mmHg and > 10 years > 90 mmHg.

Pediatric IV Fluid maintenance rate:

4 mL for the first 10 kg of weight +
2 mL for the second 10 kg of weight +
1 mL for every additional kg in weight after 20 kg

Example: 34 kg pediatric	
First 10 kg:	4 mL/kg/hr = 40 mL/hr
Second 10 kg:	2 mL/kg/hr = 20 mL/hr
Final 14 Kg:	1 mL/kg/hr = 14 mL/hr
Total: 74 mL/hr rate	
- **Bring bottles, contents, emesis to ED.**
- **S.L.U.D.G.E: Salivation, Lacrimation, Urination, Defecation, GI distress, Emesis.**
- **D.U.M.B.B.E.L.S: Diarrhea, Urination, Miosis, Bradycardia, Bronchorrhea, Emesis, Lacrimation, Salivation.**
- **Tricyclic:** 4 major areas of toxicity: seizures, dysrhythmias, hypotension, decreased mental status or coma; rapid progression from alert mental status to death.
- **Acetaminophen:** initially normal or nausea/ vomiting. If not detected and treated, causes irreversible liver failure.
- **Aspirin:** Early signs consist of abdominal pain and vomiting. Tachypnea and altered mental status may occur later. Renal dysfunction, liver failure, and or cerebral edema among other things can take place later.
- **Depressants:** decreased HR, decreased BP, decreased temperature, decreased respirations, non-specific pupils.
- **Stimulants:** increased HR, increased BP, increased temperature, dilated pupils, seizures.
- **Anticholinergic:** increased HR, increased temperature, dilated pupils, mental status changes.
- **Cardiac Medications:** dysrhythmias and mental status changes.
- **Solvents:** nausea, coughing, vomiting, and mental status changes.
- **Insecticides:** increased or decreased HR, increased secretions, nausea, vomiting, diarrhea, pinpoint pupils.
- **Nerve Agent Antidote kits** contain 2 mg of Atropine and 600 mg of pralidoxime in an autoinjector for self administration or patient care. These kits may be available as part of the domestic preparedness for Weapons of Mass Destruction.
- **EMR and EMT may administer naloxone by IN / IM route only and may administer from EMS supply. Agency medical director may require Contact of Medical Control prior to administration and may restrict locally.**
- **When appropriate contact the North Carolina Poison Control Center for guidance, reference Policy 18.**
- **Consider restraints if necessary for patient's and/or personnel's protection per the Restraint Procedure.**